DEPARTMENT OF HEALTH &. HUMAN SERVICES

Public Health Service

Indian Health Service
Phoenix Indian Medical Center
4212 North 16th Street
Phoenix, Arizona 85016

To: Christopher Davidson, Supervisory Radiologic Diagnostic Technologist
Cc: Traci Pritchard, Chief of Radiology
Cc: Charisse Holiday, Chief Professional Officer
From: Kevin Turnage, Policy and Procedure Officer, Nurse Consultant
Date: May 23, 2025
Subject: Rejection of Oral Contrast Media Administration Policy (PolicyStat ID: 18214016)

Dear Mr. Davidson,

After a thorough review of the submitted policy titled *"Oral Contrast Media Administration,"* this memorandum serves as formal notice of rejection. The policy does not meet key standards required by the **Phoenix Service Unit (PSU) Policy Management policy (PolicyStat ID 16791609)**. Below is a summary of the deficiencies, aligned with the specific language of the PSU policy.

Reasons for Rejection

1. Lack of Referenced Standards and Regulatory Alignment

Policy Language – Section F.1.e:
"Comply with current accreditation standards, higher-headquarter policy within the IHS and federal government, regulations, laws, and evidence-based best practices and guidelines."

Issue:
The current policy includes no references to clinical guidelines, regulatory requirements, or evidence-based literature to support contrast agent use, contraindications, or procedural instructions.

Required Action:
Integrate authoritative references such as FDA drug safety updates, American College of Radiology (ACR) contrast guidelines, The Joint Commission (TJC) standards, and Indian Health Service (IHS) policy to substantiate all clinical directives and enhance regulatory compliance.

2. Absence of Training and Competency Requirements

Policy Language – Sections F.1.i–k and F.2.j:
Requires that "staff are informed, trained, and competent when implementing new and revised documents."
Mandates education and attestation when documents "require a change in behavior or workflow to comply with new or revised non-regulatory content," or when "an accreditation standard or a regulation requires documentation of education."

Issue:
The policy introduces procedures with significant safety implications—such as administering contrast to patients with known or potential hypersensitivity—yet provides no framework for staff education, training, or competency assessment.

Required Action:
Include a structured training plan, specify roles responsible for delivering and validating education, and require documented attestation or post-training assessments for those administering oral contrast.

3. No Defined Management of Contrast Reactions

Policy Language – Section F.1.m:
Requires policy owners to "actively monitor for updates in evidence-based practices, clinical guidelines, regulatory requirements, and accreditation standards relevant to the content of assigned documents."

Issue:
Despite known clinical risks—such as allergic-like hypersensitivity reactions and aspiration—this policy provides no guidance for identifying at-risk patients or managing adverse events. It also lacks documentation of safe alternatives for patients with contraindications.

Required Action:
Include or clearly reference institutional protocols for:

- **Allergy risk screening** (e.g., pre-administration questionnaires or order set alerts),
- **Emergency response** (e.g., standardized anaphylaxis kits, response team activation process),
- **Safe contrast alternatives** (e.g., use of barium or Readi-Cat in iodine-sensitive patients).

These elements must be supported by up-to-date clinical guidance, such as from the American College of Radiology (ACR), FDA product labeling, and internal pharmacy or radiology SOPs. If institution-wide protocols already exist, cite them directly and indicate where they are accessed.

4. Unclear CT Dilution Instructions

Policy Language – Sections F.1.a and F.1.b:
Requires that "documents are managed with timely updates, revisions, approvals, education, and implementation," and "ensure reliable outcomes, reduce variation, and mitigate risk."

Issue:
The policy's instructions for oral contrast dilution are vague, lack quantitative specifics, and could result in clinical variability and safety concerns.

Required Action:
Clarify CT dilution protocols, include validated dosage ratios, and reference a formal standard operating procedure (SOP) to ensure consistent and safe implementation across providers and imaging modalities.

Required Revisions and Next Steps

To move forward, please revise the policy to:

- Incorporate relevant evidence-based and regulatory references.
- Establish and document training and competency validation requirements for staff.
- Add defined risk mitigation protocols consistent with current clinical standards.
- Provide detailed, standardized instructions for contrast dilution.

Once revised, resubmit the policy for expedited review. If you require assistance with interpretation of these requirements, I am available for support.

Policy Officer Disclaimer

This review was completed in accordance with the responsibilities of the Policy & Procedure Officer, Nurse Consultant, under PSU Policy Management. Final accountability for document content, compliance, and implementation rests with the designated document owner and leadership.

Respectfully,

Digitally signed by Kevin D. Turnage -S
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Hon. Kevin D. Turnage, MSN, RN, NHDP-BC, NPD-BC, AMB-BC, NE-BC
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Evidence-Based References (APA 7th Edition)

American College of Radiology. (2024). *ACR manual on contrast media*. https://www.acr.org/Clinical-Resources/Clinical-Tools-and-Reference/Contrast-Manual

American Society of Health-System Pharmacists. (2025). *ASHP injectable drug information*.

Indian Health Service. (n.d.). *Indian Health Manual (IHM), Part 3 – Professional Services*. U.S. Department of Health and Human Services. https://www.ihs.gov/ihm/pc/part-3/

Institute for Safe Medication Practices. (2020). *ISMP targeted medication safety best practices for hospitals*. https://www.ismp.org/sites/default/files/attachments/2020-02/2020-2021%20TMSBP-%20FINAL_1.pdf

The Joint Commission. (2025). *Comprehensive accreditation manual for hospitals (CAMH)*. https://store.jcrinc.com/2025-comprehensive-accreditation-manual-for-hospitals-camh-book-/cah25/

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